

Catheter-Associated Urinary Tract Infection (CAUTI) Prevention

Policy Number	POL-IC-022	Owner / Department	Infection Prevention & Control / Nursing
Effective Date	2023-08-15	Version	3.1
Last Revised	2026-02-18	Next Review	2028-02-18
Approved By	Infection Prevention & Control Committee	Applies To	All units and providers who insert, maintain, or order indwelling urinary catheters

1. Purpose

To reduce catheter-associated urinary tract infections by limiting indwelling urinary catheter use to appropriate indications, inserting aseptically, and removing promptly, consistent with the CDC/HICPAC Guideline for Prevention of Catheter-Associated Urinary Tract Infections.

2. Definitions

- **Indwelling urinary catheter** — a drainage catheter (Foley) left in place and retained by an inflated balloon.
- **CAUTI** — a urinary tract infection in a patient who had an indwelling urinary catheter in place for > 2 consecutive days on the date of the event, per the NHSN surveillance definition.
- **Nurse-driven removal protocol** — a standing order allowing nursing to remove a catheter once no appropriate indication remains, without an additional provider order.

3. Appropriate Indications and Procedure

Insert an indwelling urinary catheter only for an appropriate indication and remove it as soon as the indication resolves. Do not use indwelling catheters to manage incontinence or for staff or patient convenience.

Appropriate	Not appropriate
Acute retention or bladder outlet obstruction.	Management of incontinence in a continent or cooperative patient.
Accurate output measurement in the critically ill.	As a substitute for nursing care of an incontinent patient.
Selected peri-operative use; prolonged surgery; large-volume infusion or diuresis.	Prolonged post-operative use without a continuing indication.
Healing of open sacral or perineal wounds in an incontinent patient; required prolonged immobilization; comfort at end of life.	Obtaining a urine culture or other diagnostic test when the patient can void.

1. Perform hand hygiene and insert using aseptic technique and sterile equipment, with the smallest effective catheter.
2. Secure the catheter, maintain a sterile closed drainage system, and keep the bag below the bladder and off the floor at all times.

3. Assess necessity every shift and remove at the earliest indication under the nurse-driven protocol.
4. Do not irrigate routinely; do not change catheters or bags at fixed arbitrary intervals.

IMPORTANT — Review the necessity of every indwelling urinary catheter every shift and remove it the moment no appropriate indication remains. Duration of catheterization is the most important modifiable risk factor for CAUTI; nursing may remove under the standing protocol without a new order.

4. Severity & Reporting

Each CAUTI is classified under POL-RM-013. A CAUTI progressing to urosepsis, bacteremia, ICU transfer, or death is SEV-1 and requires notification of the attending provider, Charge Nurse, Nursing Supervisor, Risk Management, Infection Prevention, and the Administrator-on-Call immediately and within 1 hour of discovery. An uncomplicated CAUTI requiring antimicrobial treatment is SEV-2 — notify the attending provider, Charge Nurse, Risk Management, and Infection Prevention within 4 hours. A catheter kept in without a valid indication but caught before infection is a Near Miss, reported within 24 hours.

5. Documentation Requirements

- Documented indication at insertion; insertion date, time, and aseptic technique.
- Daily necessity review and date/time of removal.
- For a confirmed CAUTI: organism, catheter days, assigned severity, and all notifications; reported to NHSN by Infection Prevention.

6. References

- CDC/HICPAC, Guideline for Prevention of Catheter-Associated Urinary Tract Infections (2009).
- SHEA/IDSA/APIC, Strategies to Prevent CAUTI in Acute Care Hospitals (2022 Update).
- CDC NHSN, Urinary Tract Infection (CAUTI) surveillance definition, Patient Safety Component Manual.
- Related: POL-RM-013 (severity & reporting), POL-IC-020 (hand hygiene).